14.7 Chest Trauma (Rib Fractures and Flail Chest)*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I cannot take a deep breath because of pain." **Objective** • Paradoxical chest wall movement on one side • RR 30/min, shallow • SpO_2 89%	**ACTUAL** Ineffective breathing pattern related to chest wall instability and pain as evidenced by paradoxical movement, tachypnea and hypoxemia.	• RR 12–24/min and SpO_2 94% or above within 12 hours.	• Give oxygen as prescribed. • Sit him upright, or on the injured side if ordered. • Make sure analgesia (nerve block or epidural as ordered) is enough for deep breaths. • Follow SpO_2, RR and ABG. • Prepare for ventilation if respiratory failure develops.	• Goal reached when breathing is effective.
Subjective • Pain 8/10 on breathing **Objective** • Tender ribs • Splints his chest	**ACTUAL** Acute pain related to rib fractures as evidenced by pain on breathing and splinting.	• Pain at or below 4/10 on deep breathing.	• Give regular analgesia. • Teach him to hold a pillow to the chest when coughing. • Score pain before and after breathing exercises.	• Met when pain is controlled.
Subjective *No specific complaint* **Objective** • Hypoventilation • Retained secretions	**RISK** Risk for infection as evidenced by risk factors: hypoventilation and retained secretions (pneumonia).	• No chest infection.	• Prompt incentive spirometry every hour while awake. • Encourage coughing once analgesia works. • Watch temperature and sputum.	• Achieved if no infection develops.

*References for this care plan: [2], [14], [35], [128]. Full entries are listed in the References section of the CPMN book.